THYMOS HEALTH

A GUIDE FOR PATIENTS & CAREGIVERS

Your Autoimmune Guide

Why autoimmune care can feel fragmented - and the questions that help you find a clearer path forward.

A practical, patient-first resource for navigating symptoms, diagnosis, treatment, monitoring, and coordinated care.

- **Understand the complexity**
- **Ask sharper questions**
- **Track what matters**



WHY THIS GUIDE EXISTS

When your symptoms cross systems, your care has to connect the dots.

If you are exhausted in a way sleep does not fix, your joints ache, your gut is unpredictable, or your thinking feels foggy - yet tests keep coming back “normal” - the experience can be deeply frustrating.

Autoimmune conditions can be difficult to diagnose and manage because symptoms change over time, overlap with other illnesses, and often span multiple organ systems.

This guide explains why that complexity exists, what thoughtful autoimmune care should account for, and what questions can help you understand how your care will be diagnosed, treated, monitored, and adjusted.



This guide is intended for general education and to help you be better prepared for managing your health. It does not provide medical advice, diagnosis, or treatment. It is not a substitute for evaluation by a qualified clinician. Individual outcomes vary.

> “There is no single test, one treatment, and no specialist who can see the whole picture alone.”

AUTOIMMUNE DISEASE IS RARELY SIMPLE

One label can hide a much bigger clinical picture.

28M	~1 in 10	~80%	34%
Americans have autoimmune disease	People worldwide have autoimmune disease	Autoimmune cases are women	Cases have 2 or more autoimmune diseases

Why diagnosis can take years

Fatigue, pain, skin changes, digestive problems, neurologic symptoms, mood changes, and inflammation can overlap across dozens of diagnoses. The pattern may only become clear when symptoms, labs, medication responses, and history are viewed together over time.

- **Each specialty focuses on one organ system and associated diagnoses.**
- **“Normal” tests do not always correlate with feeling well.**
- **New symptoms, flares, and treatments reveal ever-changing patterns.**

A common patient journey

A de-identified patient in her late 50s moved through primary care, gastroenterology, rheumatology, orthopedics, and neurology for fatigue, hives, brain fog, joint pain, and IBS-like symptoms. A more coherent picture emerged only when her symptoms, chronic anemia, vitamin D deficiency, thyroid dysfunction, and elevated inflammatory markers were considered together.

The patient's issues were addressed with one coordinated multi-specialty plan spanning nutrition, micronutrient repletion, cardiovascular evaluation, gut health, lifestyle support, and longitudinal care. Within 3 months, reported symptom burden fell by 50%, and by 12 months, the patient sustained broad improvement, including resolution of brain fog, normalized lipids, weight loss, better sleep, and improved quality of life.

> “Autoimmune disease often looks less like one diagnosis with one solution and more like several interacting problems.”

WHY GOOD CARE CAN STILL BE DIFFICULT

Great doctors are often constrained by fragmented, reactive systems.

01

Multiple issues at once

Overlapping diagnoses, different treatment responses, and psychosocial influences all impact how you feel.

02

Specialists see different pieces

Rheumatology, GI, nephrology, dermatology, neurology, psychiatry, and primary care may each hold an important part of the story.

03

Care is built around visits

Health is continuously evolving. Symptoms flare, treatment response plateaus, or side effects often need immediate attention between visits.

04

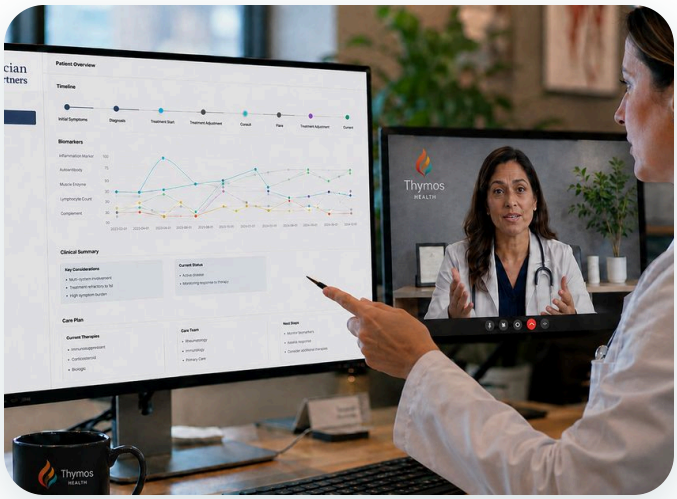
Treatment is often stepwise

Clinical guidelines and insurance requirements often create a “try, assess, then escalate” pathway that isn’t always the best option for you.

> “No single visit, and often no single specialist, is designed to understand the whole picture.”

TREATMENT IS A PROCESS, NOT A PRESCRIPTION

Your condition changes. Your plan should be able to change with it.



Autoimmune symptoms can shift with infection, stress, sleep, hormones, nutrition, environmental exposures, medications, and the underlying disease itself.

The practical question is not only **“What treatment are we starting?”** It is also **“How will we know quickly whether it is working for me?”**

> “Care doesn't pause between scheduled appointments.”

Four things to define up front

- ✓ What success should look like
- ✓ Which symptoms and biomarkers matter most
- ✓ When response will be reassessed
- ✓ What triggers a dose change, add-on, switch, or new workup

Observe
Symptoms, labs, function, side effects

→

Interpret
Patterns against your history and goals

→

Adjust
Change the plan when the evidence changes

A CONTINUOUS LOOP - THE PLAN KEEPS ADJUSTING AS YOUR CONDITION CHANGES

You deserve care that looks at the full picture - not just the next symptom.

QUESTIONS WORTH ASKING YOUR DOCTOR

These questions will help you proactively manage your care.

These questions are not meant to challenge your physicians. They help make expectations, monitoring, ownership, and timelines explicit.

ABOUT THE TREATMENT

What outcome should I realistically expect, and what evidence supports this choice for someone with my history?

ABOUT TIMING

How long should it take before we know whether this is working?

ABOUT MONITORING

Which symptoms, biomarkers, or measures will be tracked - and how often will they be reviewed?

BETWEEN VISITS

How will my case be monitored between appointments? How do I reach you if my symptoms change, and how quickly will you respond?

PERSONALIZATION

How does this plan differ from the standard protocol for someone like me?

WHOLE-PERSON CONTEXT

How are my other diagnoses, medications, symptoms, lifestyle, and goals being considered?

ESCALATION

At what point would you decide this treatment isn't working, and how quickly do you move to the next line of therapy?

OWNERSHIP

Who is responsible for noticing if I am not getting better - and what happens next?

DEFINING SUCCESS

What should be different three months, six months, and one year from now if our plan is working?

WHEN TO RETHINK

What would make you change the plan entirely rather than continue adjusting it?

One whole-patient view. One physician-led plan. Continuous reassessment.

Thymos Health was built for patients whose condition has become complex, fragmented, or difficult to reverse. The model is designed to combine information that often lives in separate records, specialties, and visits.

1 **Reconstruct**
Organize years of symptoms, diagnoses, tests, medications, and specialist visits into one clinical timeline.

2 **Contextualize**
Interpret findings against the full patient picture, including history, disease activity, medications, and relevant lifestyle or environmental factors.

3 **Personalize**
Build a treatment strategy around the patient's diagnoses, goals, treatment response, and clinical priorities.

4 **Reassess**
Track symptoms and clinical data over time so the plan can evolve as the patient's condition changes.



Whole-system autoimmune specialist

Medical history · Diagnostics · Symptoms · Medications · Nutrition · Microbiome · Stress · Environment

Care between visits is part of treatment

Through the Thymos app, patients can track symptoms, receive personalized support, and communicate with their care team. The goal is to make clinically relevant changes visible sooner, rather than waiting for the next routine appointment.

>80%
of patients experienced symptom remission by month three in Thymos' internal real-world outcomes study

9/10
patient satisfaction score reported in Thymos internal program materials

Internal Thymos Health real-world outcomes across more than 5,000 treated patients. These results have not been independently validated in the source material provided for this guide. Individual results vary and should not be interpreted as a guaranteed outcome. If you are experiencing a medical emergency, call 911 or seek acute medical care.

Selected sources

Scientific American. “Why Nearly 80 Percent of Autoimmune Sufferers Are Female.”
Mayo Clinic News Network. U.S. autoimmune disease prevalence study summary.
CreakyJoints. Research on delays and misdiagnosis in autoimmune disease.
American Journal of Gastroenterology (2023). Abstract on IBD misdiagnosis.
National Organization for Rare Disorders. Step therapy / “fail first” policy overview.
Magro et al. Journal of Crohn's & Colitis (2013); Wang et al. BMC Gastroenterology (2021).
Additional citations and source links are preserved in the original Thymos guide draft.